



Olanzapine Long Acting Injections (LAI) SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RBPB)	Mental Health Division (MHD) Community Mental Health Services (CMHS) (Midland, Bentley, City East and Wungen Kartup)	Nursing, Pharmacy and Medical

Purpose

To assist clinicians in the safe prescription, administration and monitoring of olanzapine pamoate (Zyprexa Relprevv®) intramuscular (IM) injection of consumers in community settings.

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Background

Olanzapine pamoate monohydrate (Zyprexa Relprevv®) is:

- **A long acting injectable (LAI)** form of medication given every 2 to 4 weeks for the maintenance treatment of schizophrenia
- A crystalline salt formulation that is suspended in water. When injected into muscle, the crystals slowly dissolve and dissociate, releasing olanzapine LAI
- To be prescribed and administered within the CMHS clinics due to monitoring requirements and in accordance with the manufacturer's product information and [Guidelines for the Use and Management of Long-acting Injectable Antipsychotics in Serious Mental Illness](#)¹

Caution: Olanzapine pamoate monohydrate is different to olanzapine (Zyprexa IM®), which is a **short acting IM injection used** for acute agitation and arousal.

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Definitions

Hypersensitivity	Extreme physical sensitivity to particular substances or conditions.
Long Acting Injection (LAI)	Intramuscular antipsychotic injections with a prolonged effect resulting in the slow release of the active principle or the continued absorption of small amounts of the dosage of the medication over an extended period.
Post Injection Syndrome (PIS)	Signs and symptoms of olanzapine overdose due to inadvertently injecting olanzapine pamoate monohydrate into the blood stream. One or more of the signs and symptoms may develop initially within 1 hour of administration and resolve within 24 – 72 hours.³ Signs and symptoms of PIS following injection of olanzapine LAI include: • Sedation • Confusion • Agitation/anxiety • Aggressiveness • Dizziness • Ataxia • Extrapyramidal symptoms (EPS) • Convulsions NB: PIS may occur following the administration of olanzapine LAI despite no prior problematic effects at previous injections.
Verbal Consent	Where a verbal (compared with written) agreement to a treatment or procedure is discussed with and agreed to by a consumer.

Roles and responsibilities – Administering Clinician (e.g. Wellness Clinic Nurse / Nursing member of the treating team/ Care Coordinator)

Refer to [Appendix III](#).

Procedure details

The following is to be noted in regard to the administration and monitoring of olanzapine LAI:

- Staff must be skilled in medication administration of this medication and have knowledge of potential adverse reactions
- The clinician administering the medication is responsible to do so within their scope of practice
- Nurses must be Registered Nurses (RNs) or Enrolled nurses and confident in the preparation and use of this agent
- Clinicians are to raise any concerns about the administration of this medication with the relevant line manager and/or prescriber
- Administration will be in accordance with the [Guidelines for the Use and Management of Long-acting Injectable Antipsychotics in Serious Mental Illness](#)¹

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Inclusion criteria

Referrals for olanzapine LAI is suitable for the following consumers:

- Aged 18 years and above
- Who have responded well to oral olanzapine
- Who are non-compliant with oral medication or who cannot guarantee adherence or who have a preference to LAIs
- Who are able to tolerate test doses of oral olanzapine to ensure that no hypersensitivity reaction occurs prior to administration of olanzapine LAI. This responsibility resides with the treating Medical Practitioner (e.g., Medical Officer [MO]/Duty MO and documented in the integrated notes and Psychiatric Services Online Information System (PSOLIS)
- Acceptance of:
 - Pre injection assessment or screen and 2-hour post monitoring/observation period and who have transport to their destination after each injection
 - That the consumer is not to drive (e.g., car, motor bike) or operate heavy machinery for the remainder of the day after each injection
 - Remaining in the CMHS clinic for at least 2 hours for post injection monitoring ([MIMS Product Information](#)). Refer to monitoring requirements section
 - Monitoring time extension if any side effects occur.

NB: If a Consultant Psychiatrist considers that the benefit of prescribing olanzapine LAI to a non-compliant consumer (e.g., those refusing to remain 2 hrs in the clinic for post injection monitoring) outweighs the risks associated with non-compliance of post monitoring, they must document this in the consumer's integrated notes when prescribing olanzapine LAI. To be reviewed prior to each administration/injection.

Exclusion criteria

Referrals for olanzapine LAI is not suitable for the following consumers:

- Refuse service/are uncooperative* see [textbox above](#)
- Previous non-compliance with the required monitoring, whether or not the consumer has signed the relevant section of the *Olanzapine LAI Post-Observation Sheet*
- Over 75 years of age and under 18 years of age.

Special precautions for the use of olanzapine LAI

- It is not recommended for treatment for older adults (over 65 years) unless a well-tolerated and effective oral dose regime has been stabilised. A lower starting dose is to be considered (150mg every 4 weeks)
- It is not licensed for dementia-related psychosis and/or dementia related behavioural disturbance for consumers under 18 years of age
- It is to be used in consumers with renal and/or hepatic impairment only if a well-tolerated and effective oral dose regime has been established. A lower starting dose is to be considered (150mg every 4 weeks).

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Prior to LAI administration

Warning: The potential for signs and symptoms of sedation and/or delirium consistent with olanzapine overdose exists after every injection of olanzapine pamoate monohydrate. Olanzapine pamoate monohydrate should be administered by appropriately qualified clinicians and consumer monitored at least two (2) hours after each injection.

The administering clinician is to:

- Obtain verbal, consumer consent and document in the *integrated notes*
- Advise the consumer of the requirement to stay in the clinic for post depot monitoring for 2 hours and agree to these terms before the depot is administered. See below section 'Refusal to Engage' (pg.5), if relevant, and document in the consumer's *integrated notes*
- Advise the consumer that they should not drive or operate heavy machinery for the remainder of the day after each injection
 - Depending on the individual circumstances of the consumer, advise the consumer that transportation assistance will be considered for consumers who cannot be conveyed to and from the identified service by a third party due to the driving restriction conditions post administration and document in the integrated notes. Refer to **RPMG Transport – Hospital and Voluntary Policy**
- Administering clinician is to provide/offer consumer:
 - written information on olanzapine LAI ([WA Department of Health CHOICE Medication leaflet](#)) to support verbal counselling
 - text appointment date/time message
 - olanzapine appointment card with symptoms to look out for and emergency contact information (as per [Appendix II](#)) and document this in the progress notes and PSOLIS
- Inform the consumer of
 - risks versus benefits, including the risk of post injection syndrome (0.7% of injections or 1.85% of consumers) (see [MIMS Product Information](#)⁴)
 - what action will be taken if side effects occur, including assessment by a MO/DMO and may include attendance at an Emergency Department (ED)
- Confirm agreement from consumer that monitoring time will be extended if side effects occur and document in the consumer's integrated notes
- Ensure that the requirement and agreed plan for olanzapine LAI administration forms part of the consumer's management plan
- Schedule olanzapine LAI treatment during the usual CMHS clinic opening times to ensure sufficient time for monitoring requirements to be completed, where possible. Individual services may decide their own specific day/time suitable to provide olanzapine LAI administration to consumers.

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Administration in the CMHS Clinic

The administering clinician is to:

- Administer and monitor olanzapine LAI to eligible consumers in the CMHS clinic treatment room
- Consider arranging the administration and/or post monitoring duties to be provided by an alternative staff member who is suitably trained
- Where possible, schedule treatment to occur during the usual CMHS clinic opening times to ensure staff availability so that enough time is allocated for monitoring to be completed
- Document in the consumer's integrated notes and PSOLIS that the consumer has been advised of post depot monitoring and driving/operating heavy machinery conditions/restrictions. Any evidence that suggests a consumer is not adhering to the essential conditions of olanzapine LAI administration will trigger a review by the CC and allocated Medical Practitioner. The outcome of the review will be communicated to the consumer and documented in the integrated notes and/or management plan and PSOLIS.

Prescribing

The prescribing clinician is to:

- Provide consumers written information on olanzapine LAI at commencement of treatment (offer [WA Department of Health CHOICE Medication leaflet](#)) and relevant information (where required) at every clinical review and document this discussion in the integrated notes
- Check consumers have a recorded history of response and tolerability to oral olanzapine to ensure no hypersensitivity reaction. DO NOT prescribe IMI to olanzapine naïve consumers
- Inform consumers of the side effect profile of olanzapine LAI including the risk (0.07%) of a PIS event (see [MIMS Product Information](#)⁴)
- Refer to recommended doses of olanzapine LAI shown in Table 1
- Ensure full review of consumer's eligibility for continued treatment with olanzapine LAI relating to any refusal to adhere with monitoring requirements.

Refusal to engage

The administering clinician is to escalate and inform the treating team for all consumers not adhering to the mandatory conditions of olanzapine LAI administration/monitoring

- Consumers who refuse to have the injection or the required monitoring (i.e. are non-compliant) will be escalated to the Treating Consultant/Duty Consultant. In such circumstances, the Treating Consultant/Duty Consultant is to be involved in considering continuation/change to alternative depot medication and this must be documented in the progress notes and form the basis of the individual consumer's management plan

Administration in the CMHS Clinic – Refusal to engage cont'd

- If a consumer leaves the CMHS clinic prior to the completion/adherence of the monitoring period:
 - Administering clinician to fully inform the consumer of the responsibilities in leaving the clinic against medical advice. Document events and actions taken by staff in the integrated notes and PSOLIS, including where contact with the consumer/ personal support person (PSP)/support worker is subsequently made
 - Escalate to the treating Consultant/Duty Consultant and medical review if the consumer doesn't sign/complete the signed section of the Olanzapine LAI Post-Observation Sheet. Compliance is audited as per the RPBG Performance Monitoring Schedule
 - Treating Team to complete a clinical review prior to the next depot appointment and document the outcome in the progress notes and PSOLIS.

If a consumer leaves the clinic without signing (or refuses to sign) the relevant section of the *Olanzapine LAI Post-Observation Sheet*:

- The administering clinician will immediately inform the Treating Consultant/Duty Consultant, who will complete a review and establish a plan to ensure compliance with required monitoring prior to the next planned administration date
- The Treating team to communicate the outcome of the review, inform the consumer and administering clinician and document in the consumer integrated notes, management plan and PSOLIS
- Administering clinician will make all reasonable attempts to
 - contact the consumer to advise of the possible consequences of not adhering to the monitoring guidelines and to encourage the consumer to return to the CMHS clinic to complete the post-depot monitoring supervision requirements
 - if a consumer declines to return to the clinic or attempts to contact consumer are unsuccessful
 - contact the consumer's next of kin (NoK)/PSP/ guardian where consent provided and advise of the information above
 - document the communication attempts (both successful and unsuccessful) in the consumer's integrated notes and in PSOLIS
 - escalate unsuccessful attempts to contact the consumer and/or NoK/PSP to the consumer's allocated Care Coordinator (CC)/MO/DMO or Duty Clinician to follow-up with medical staff and document the outcome in the consumer's integrated notes and PSOLIS
 - CC to organise a home visit to do a welfare check. If no answer, CC to leave a note to contact clinic or after hour (A/H) team and hand over to A/H team to follow up
- If the administering clinician fails to notify Consultant/Duty Consultant /MO/DMO or the treating team neglects to communicate with the consumer, clinicians identifying issue to complete a Datix Clinical Incident Management System (CIMS).

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Administration in the CMHS Clinic cont'd

Dosage and administration

Formulation of olanzapine LAI includes 3 strengths: 210 mg; 300 mg; and 405 mg.

- Oral olanzapine (immediate release) should be trialled in consumers prior to commencement of olanzapine LAI
- Olanzapine LAI is administered via deep IM injection into the ventrogluteal or dorsogluteal muscle. Refer to **Medication Preparation and Routes of Administration Clinical Guideline**
- See Table 1 for reconstitution instructions
- Once reconstituted, the product should be used as soon as possible. The reconstituted product should be stored for not more than 6 hours at room temperature (25 °C) before being used
- The plasma half-life of olanzapine after administration of the LAI is 30 days.

Note: While plasma levels have usually diminished considerably after 8-12 weeks, elimination of olanzapine may not be complete until 6-8 months after the last injection.

- Supplementation with oral olanzapine is not required at the start of the treatment with olanzapine LAI.

Table 1: Reconstitution Instructions

Dose	Olanzapine pamoate vial strength	Volume of diluent to add (mL)	Final volume to inject (mL)
150mg	210mg	1.3mL	1.0mL
210mg	210mg	1.3mL	1.4mL
300mg	300mg	1.8mL	2.0mL
405mg	405mg	2.3mL	2.7mL

Note: there will be left over diluent and solution in vial.

Monitoring requirements

Prior and post administering Olanzapine LAI

The administering clinician is to:

- Complete a full set of clinical observations and document in the olanzapine LAI Pre *Observation Sheet* ([Appendix I](#)) and monitoring (pre and post-observation) in the CMHS clinic section of the consumer's integrated notes and PSOLIS
 - Should a consumer decline a full set of clinical observations pre-injection the administering Clinician is not to proceed with administering olanzapine LAI and escalate to the Treating Consultant/Duty Consultant for further direction
- Monitoring the consumer at 30min intervals for a minimum of 2 hours. Document findings on *Olanzapine LAI Pre Observation Sheet*
 - Level of alertness
 - Orientation
 - Any signs and symptoms of Post Injection Syndrome

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Administration in the CMHS Clinic – Monitoring requirements cont'd

- Prior to the consumer leaving the CMHS clinic, ensure the consumer
 - is alert, oriented with no signs and symptoms of Post Injection Syndrome and document this outcome in the integrated notes and PSOLIS
 - include post olanzapine LAI monitoring in the Treatment Support Discharge Plan
 - completes the signed section of the olanzapine LAI Post-Observation Sheet ([Appendix I](#)), including consumer name, signature and time of departure
 - has information on olanzapine LAI and/ offer, if required- [WA Department of Health CHOICE Medication leaflet](#) and provide olanzapine appointment card ([Appendix II](#)) and text appointment message (if relevant) and document this in the progress notes and PSOLIS

Consumers with signs and symptoms of Post Injection Syndrome

For consumers who develop or show signs or symptoms of Post Injection Syndrome, the administering clinician is to:

- increase frequency of clinical observations to 15-minute intervals and request an immediate review by a MO/DMO. Document observations on *AORC chart*
- monitor the consumer for an additional 1 hour or until symptoms resolve
- MOs/DMOs to review consumer to either clear the consumer for discharge or approve transfer to ED if indicated
 - if ED transfer is required, contact Emergency Physician in Charge (EPiC) and provide handover
- **activate MET** (Medical Emergency Team) as per criteria or call 000 (for non- hospital sites) immediately if vital signs **become unstable** – use the *Olanzapine LAI Pre- and Post-Observation Sheet* and refer to the **R PBG Medical Emergency and Cardiorespiratory Arrest Management CPS**
- complete a Datix CIMS if a consumer experiences Post Injection Syndrome or any, other adverse effects before monitoring is completed and document this in the consumer integrated notes and PSOLIS.

Missed dose

- Assess consumer, contact MO/medical team for review and document in the integrated notes and PSOLIS and inform CC
- Contact Pharmacist or MO/medical team for clinical advice and/or dose adjustment.

Compliance monitoring

Documentation of monitoring requirements will be monitored and audited in accordance with the R PBG Performance Monitoring Schedule for the individual service and by a review of clinical incidents via Datix CIMS related to olanzapine treatment.

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Feedback received

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References

1. Llorca, P.M.; Abbar, M.; Courtet, P.; Guillaume, S.; Lancrenon, S.; and Samalin, L. (2013). *Guidelines for the use and management of long-acting injectable antipsychotics in serious mental illness*. BMC Psychiatry., v.13: 340. USA. Published online Dec 20 2013 doi: [10.1186/1471-244X-13-340](https://doi.org/10.1186/1471-244X-13-340)
2. Mind UK <https://www.mind.org.uk/information-support/drugs-and-treatments/antipsychotics/depot-injections/> accessed 24/04/2023
3. Luedecke D, Schöttle D, Karow A, Lambert M, Naber D. Post-injection delirium/sedation syndrome in consumers treated with olanzapine LAI pamoate: mechanism, incidence, and management. CNS Drugs. 2015 Jan;29(1):41-46. DOI: 10.1007/s40263-014-0216-9. PMID: 25424243.
4. [MIMS Product Information](#)

Related policy, practice standard, clinical guidelines

[RPBG Policy Hub](#)

- [Medical Emergency and Cardiorespiratory Arrest Management \(MET\) CPS](#)
- [Transport Policy – Hospital Patient and Voluntary Transport Services](#)
- [Medication Preparation and Routes of Administration CPS](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Escalation to Consultants Policy](#)

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Related policy, practice standard, clinical guidelines cont'd

[EMHS Policy Hub](#)

- [Consent Policy](#)

[Department of Health Policy Hub](#)

- [High Risk Medication Policy](#)
- [Medication Review Policy](#)
- [Medicines Handling Policy](#)
- [Consent to Treatment Policy](#)
- [Clinical Incident Management System Policy](#)
- [State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)

Other

- [WA Department of Health CHOICE Medication leaflet](#)
- [National safety priorities in mental health: a national plan for reducing harm 2005](#)
- [National Medicines Policy 2000](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Clinical Guidelines for the Physical Care of Mental Health Consumers](#)
- [Intramuscular Administration of Long-Acting Antipsychotics](#)
- [Poisons Regulations June 2016](#)
- [Mental Health Act \(MHA\) 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)

Related forms

- *RPBG Olanzapine Long Acting Injection (LAI) Pre- and Post-Observation Sheet*
- *Adult Observation & Response Chart (AORC)*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021): Standard 4: Medication Safety; Standard 6: Communicating for Safety; Standard 8: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010): Standard 2: Safety; Standard 6: Consumers; Standard 10: Delivery of Care

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Appendix I: RPBG Olanzapine LAI pre- and post-observation sheet

ROYAL PERTH BENTLEY GROUP		OLANZAPINE LONG ACTING INJECTION PRE-OBSERVATION SHEET	
PRINT IN BLOCK LETTERS ONLY	WARD/CLINIC _____	AFFIX LABEL HERE	
		SURNAME _____ MED. REC. NO. _____ GENDER _____	
	CONSULTANT _____ R.M.O./REG. _____	FORE NAMES _____	BIRTH DATE _____
Date: _____ ☐ Received verbal consent prior to injection			
Time: _____			
Vital Signs / Observations (Refer to the RPBG Olanzapine Long Acting Injection (LAI) SOP)			
Blood Pressure:			
Standing _____		Temperature: _____	
Sitting _____		Respiration: _____	
Pulse / Heart Rate _____		Blood Glucose Levels (non fasting) _____	
Weight _____		Oxygen Sats (SpO ₂) _____	
Physical Health			
Alcohol and/or illicit substances (last 24 hours) _____			
Physical Health Concerns _____ (pain, diarrhoea, etc)			
Sleep hygiene / routine _____			
Time of last meal _____ Time of last fluids _____			
Level of Consciousness			
Glasgow Coma Scale (GCS)			
Eyes Opening		Verbal Response	Motor Responses
Spontaneously	4	Correctly gives name, place and date	5
To Speech	3	Confused	4
To Pain	2	Inappropriate words	3
None	1	Incomprehensible sounds	2
		None	1
		Extension	
		None	
Score: 14 - 15 = normal function			
Score: 11 - 13 = moderate to severe dysfunction			
Score: 10 or less = severe dysfunction			
		pre GCS	post GCS (2 hours)
		Total: _____	Total: _____
Other Comments / Review:			
Name: _____ Signature: _____ Designation: _____			

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Appendix I: RPBG Olanzapine LAI pre- and post-observation sheet cont'd

ROYAL PERTH BENTLEY GROUP				OLANZAPINE LONG ACTING INJECTION POST-OBSERVATION SHEET							
WARD/CLINIC				AFFIX LABEL HERE							
				SURNAME		MED. REC. NO.		GENDER			
CONSULTANT		R.M.O./REG.		FORE NAMES			BIRTH DATE				
(Refer to the RPBG Olanzapine LAI Long Acting Injection SOP) Date: _____ Time (injection given): _____ Time (finish): _____ Nurse to document time and initial each check											
Symptoms \ Time											
No Issues											
Weakness / malaise											
Dizziness											
Sedated											
Confused											
Slurred Speech											
Agitation / anxiety											
EPS (Extrapyramidal Symptoms): tremors, muscle jerk, akathisia, etc											
Staff initial											

DO NOT PRINT

If symptoms occur, increase to 15 min observations, inform treating doctor and administering clinician and complete vital signs and post GCS.

Post Injection Monitoring

- Common Symptoms:
Often consumers report "not feeling well" or "feeling sleepy / sluggish" (initial symptoms).
Sedated / drowsy, dizziness, weakness / malaise, confusion, agitation / anxiety, slurred speech, aggressiveness, extrapyramidal symptoms.
- Monitoring:
Monitor for the above symptoms every 30 minutes, for a total of 2 hours. The response should be recorded on this observation sheet. The staff member performing the observation should initial the appropriate column.

☐ Consumer is alert & orientated with no signs / symptoms of post-injection syndrome (PIS) prior to leaving the clinic
Advise consumer that they should not drive or operate heavy machinery for the remainder of the day after each injection:
☐ Yes ☐ No ☐ N/A

☐ Consumer provided card with symptoms to look out for and informed not to be alone for at least the next 24 hours

☐ Consumer declined monitoring Notify Treating Consultant / Duty Consultant

☐ Consumer advised of the required escalation to Treating / Duty Consultant or medical review

Escalated to Treating / Duty Consultant or requested medical review: ☐ Yes ☐ No ☐ N/A

I _____ hereby take full responsibility in leaving the clinic against medical advice.

Consumer Signature _____ Time consumer left _____

Name: _____ Signature: _____ Designation: _____

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Appendix II: Olanzapine LAI appointment card

Front

Olanzapine appointment card – clinic name & location

You have recently had an olanzapine injection.

If you experience any of the below symptoms in the next 24 hrs seek medical assistance and/or present to ED

- Feeling dizzy/weak/confused/disorientated/drowsy
- Feeling anxious/irritable/aggressive
- Having trouble talking/walking
- Having muscle stiffness/ shaking or convulsions



Government of Western Australia
East Metropolitan Health Service

Royal Perth Bentley Group

Back

Community Mental Health Service

Mental Health Emergency Response Line (MHERL)

24hrs/After hours service: 1300 555 788

Ambulance: 000

Date injection given:

Date injection next due:

.....

.....

Do not drive (e.g. car, motor bike) or operate heavy machinery for the remainder of the day after each injection



Government of Western Australia
East Metropolitan Health Service

Royal Perth Bentley Group

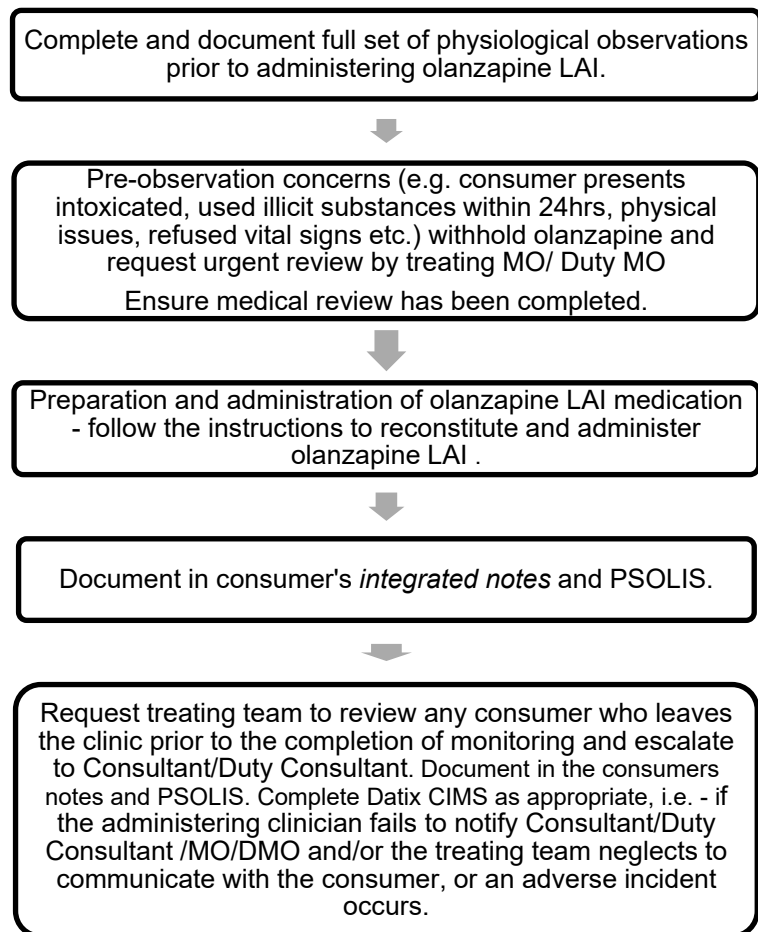
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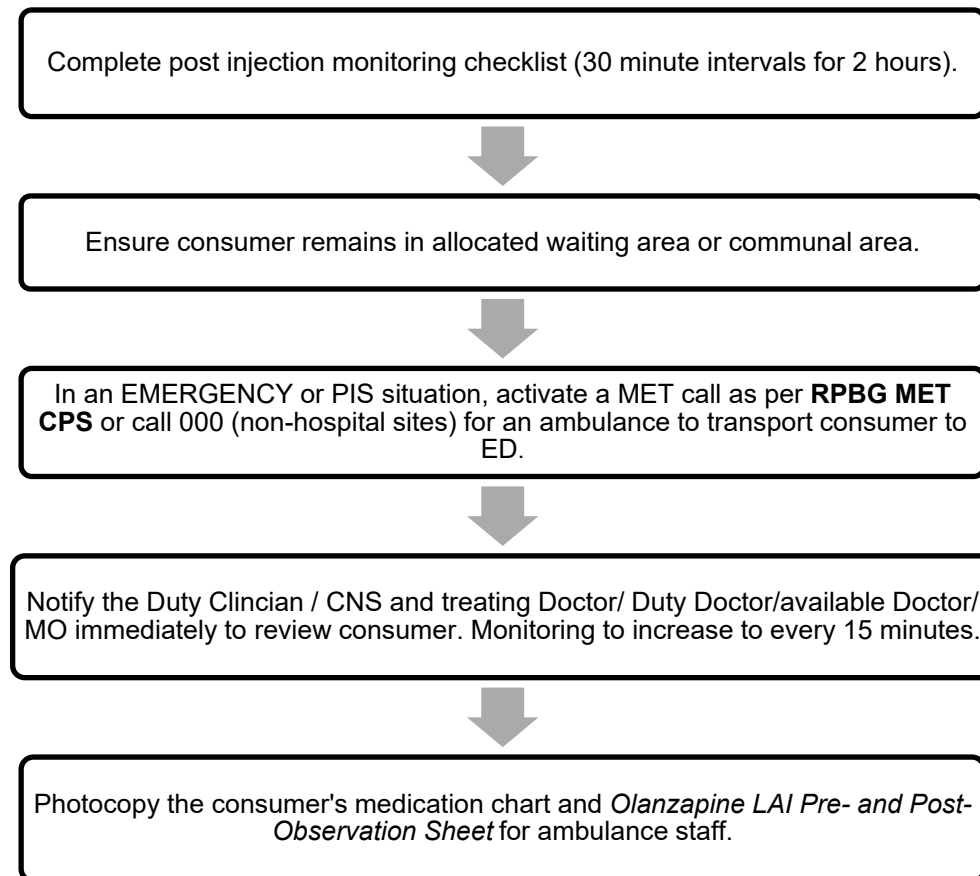
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Appendix III: Roles and responsibilities- Administering Clinician flowchart

Administering clinician



Post monitor/olanzapine LAI trained clinician



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